

as possible to build the house, and every extra dime went toward materials. He was also doing a lot of the work himself, so every evening and weekend were tied up.

With Rhonda getting fired from her job and getting progressively less able to work, Phil began to get fanatical about money. Rhonda couldn't even buy herself clothes without Phil ranting about it for days.

Rhonda began to go to various doctors for tests to determine her physical problems, and Phil exploded when the bills started coming. As their relationship deteriorated, Rhonda's health got worse. She was convinced that she had a terminal illness, or at least some exotic disease, but every test came back negative.

Her loss of income delayed the completion of their home, and the bank carrying the construction loan began to pressure them. Rhonda began to decline physically until she reached a stage of almost constant fatigue. She would wake up in the mornings as tired as when she went to bed. By afternoon, she felt so groggy that she couldn't concentrate, so she retreated to bed.

Her doctor put her through several tests and a complete physical, but could find nothing wrong with her. After a brief consultation, he prescribed some antidepressants and recommended that she seek psychiatric help.

Rhonda, a Christian, was shaken to her inner core. She realized then that she (and Phil) had allowed finances to assume first place in their lives. The urgent things (a house in this case) had replaced the important things. She asked Phil to go for counseling with her, which he reluctantly agreed to do. Their pastor then referred them to our counseling office.

After listening to the entire scenario, my recommendation was to put the house up for sale—as is—recover their investment, and then go find a home that was complete and would fit their budget.

Phil objected, obviously. He felt a personal attachment to his project, which had long since grown beyond just being a place to live. My question to Phil is one that each of us